

Infection Prevention and Control Standard Operating Procedure

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Owner	Infection Prevention and Control Program		
Approved by	Infection Control Committee		
Applies to	All inpatient units, emergency departments and clinics of Riverside Health Network		

1. Purpose

This procedure describes the practices that prevent the spread of infection among patients, visitors and staff at Riverside Health Network: hand hygiene, standard and transmission-based precautions, device bundles, environmental cleaning and outbreak response. It applies to Riverside Memorial Hospital (F01), Lakeview Community Hospital (F02), Eastgate Regional Hospital (F03) and the twelve affiliated clinics.

2. Hand hygiene

Staff clean their hands with alcohol-based hand rub, or with soap and water when hands are visibly soiled, at five moments: before touching a patient; before a clean or aseptic procedure; after body fluid exposure risk; after touching a patient; and after touching the patient's surroundings. Soap and water is always used after caring for a patient with known or suspected *Clostridioides difficile* infection, because alcohol does not kill spores.

Hand hygiene is audited monthly on every unit by trained observers, with at least 30 observations per unit. A unit below 90% compliance receives a coaching visit within 14 days and is re-audited the following month.

3. Standard and transmission-based precautions

Standard precautions apply to every patient. The table lists the added precautions for common conditions. A precaution sign is posted at the room door before the patient arrives on the unit, and the isolation flag is set in the patient's electronic record.

Condition or organism	Precaution	Required PPE	Discontinue when
MRSA or VRE, colonized or infected	Contact	Gown and gloves on room entry	Per Infection Prevention review; at least 3 negative screens for MRSA decolonization
<i>Clostridioides difficile</i>	Contact plus soap-and-water hand hygiene	Gown and gloves; dedicated equipment	48 hours after diarrhea has resolved
Influenza	Droplet	Surgical mask on room entry; eye protection if splash risk	7 days after onset or 24 hours after fever resolves, whichever is later
Pulmonary tuberculosis, suspected or confirmed	Airborne	N95 respirator (fit tested); negative-pressure room	Three negative sputum smears and clinical improvement, with Infection Prevention approval
Measles or varicella	Airborne	N95 respirator; negative-pressure room; only immune staff	4 days after rash onset (measles); until all lesions crusted (varicella)

Condition or organism	Precaution	Required PPE	Discontinue when
Norovirus	Contact	Gown and gloves; soap-and-water hand hygiene	48 hours after symptoms resolve

3.1 Patients with heart failure or COPD

Patients admitted with respiratory symptoms are screened for influenza and other respiratory viruses on arrival. A patient with COPD exacerbation and suspected influenza is placed on droplet precautions until the result is back, and no aerosol-generating nebulizer treatment is given in a shared room.

4. Device bundles

4.1 Central line (CLABSI prevention)

- Insert using maximal sterile barrier precautions and chlorhexidine skin preparation.
- Review the need for the line every day on rounds and remove it as soon as it is not needed.
- Scrub the needleless connector for 15 seconds before each access.
- Change the dressing every 7 days for transparent dressings and immediately if soiled, loose or damp.
- Bathe adult ICU patients daily with chlorhexidine.

4.2 Urinary catheter (CAUTI prevention)

- Insert only for an approved indication (acute retention, strict output monitoring in critical illness, surgery, or pressure injury healing support) and document it.
- Keep the collection bag below the bladder and never on the floor.
- A nurse-driven protocol removes the catheter within 24 hours of the indication ending.

5. Environmental cleaning

Area	Frequency	Method
Patient rooms, high-touch surfaces	Daily, and at every shift change in isolation rooms	Hospital-grade disinfectant, contact time as labeled
Rooms of patients with <i>C. difficile</i>	Daily, and at discharge	Sporicidal agent (bleach-based)
Emergency department bays	After every patient	Disinfectant wipe-down and linen change
Clinic exam rooms	Between patients	Disinfectant wipe-down of table, chair and counters
Shared equipment (scales, monitors, wheelchairs)	Between patients	Disinfectant wipe

6. Outbreak response

A cluster is two or more patients on one unit with the same organism or syndrome within 7 days. Any staff member who suspects a cluster calls Infection Prevention at once. Infection Prevention then:

1. confirms the cluster and starts a line list within 4 hours;

2. notifies the hospital's chief nursing officer and the facility leader the same day;
3. places affected patients on the right precautions and cohorts them where possible;
4. reports to the public health department when the condition is reportable, within the time the department sets;
5. holds an outbreak meeting within 24 hours and every working day until no new case has been found for two incubation periods.

7. Surveillance and reporting

Infection Prevention counts central line-associated bloodstream infections, catheter-associated urinary tract infections and hospital-onset *C. difficile* each month for each of the three hospitals and reports the standardized infection ratio to the Infection Control Committee every quarter. Units with a ratio above 1.0 for two quarters in a row write an improvement plan within 30 days.

8. Staff health

Staff with vomiting, diarrhea, a fever or a new rash do not work with patients until cleared by Occupational Health. Annual influenza vaccination is required for all staff, with documented exemptions reviewed by Occupational Health.

Riverside Health Network is a fictional organization. This document is synthetic and is not clinical guidance.